BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

Check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver	Baseline tests and dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) status	Before each 14-day cycle	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Heart	electrocardiogram (ECG) if compromised cardiac function	Report chest symptoms	Chest pain: urgent assessment	Further tests if indicated	Follow any cardiac problem

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00660 v2b official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Fluorouracil + folinic acid every 14 days: treatment and checks

Adjuvant or advanced colorectal cancer

Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00660	Baseline tests and DPD status	Before each 14-day cycle	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00660	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00660	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Heart NCCP 00660	ECG if compromised cardiac function	Report chest symptoms	Chest pain: urgent assessment	Further tests if indicated	Follow any cardiac problem

What each check is for

1 Confirm your personal plan

Before starting

Treatment after colorectal cancer surgery can reduce recurrence risk. For advanced cancer, treatment aims to control the disease. Your team confirms which aim applies.

Ask / **act:** Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00660

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32